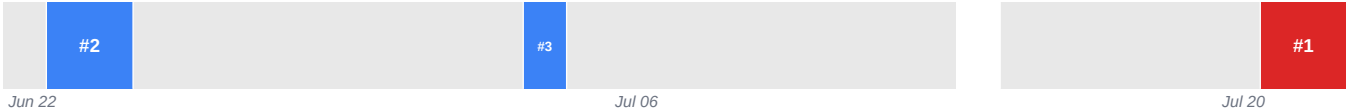


Patient ID: NancyPeriod: June 22 to July 22, 2024Report Date: July 23, 2024Coverage: 417/738h (56.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Jun 22 to Jul 22

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	10h	Low Heart Rate	<1	57%

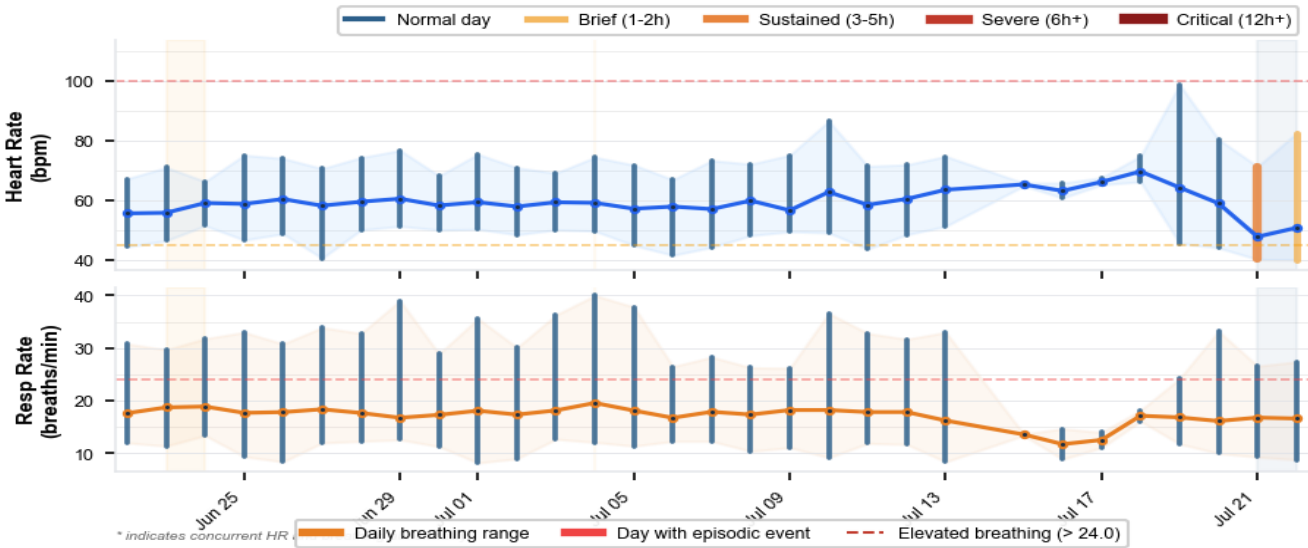
Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	40 bpm	7h	1h	4	49 bpm	Jul 21 to Jul 22
2	Elevated Breathing	28 brpm	2h	1h	1	19 brpm	Jun 23 to Jun 24
3	Elevated Breathing	25 brpm	1h	1h	1	20 brpm	Jul 04

The P5 to P95 heart rate spread was 21 bpm (48 to 69), indicating significant cardiac variability.

Clinical Pattern Observations:

- 1. **High variability:** HR spread 20 bpm (47 to 68).
- 2. **Clustered pattern:** Episodic events on 2 of 31 days (6%).
- 3. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.



Red bars indicate days with detected episodic events.

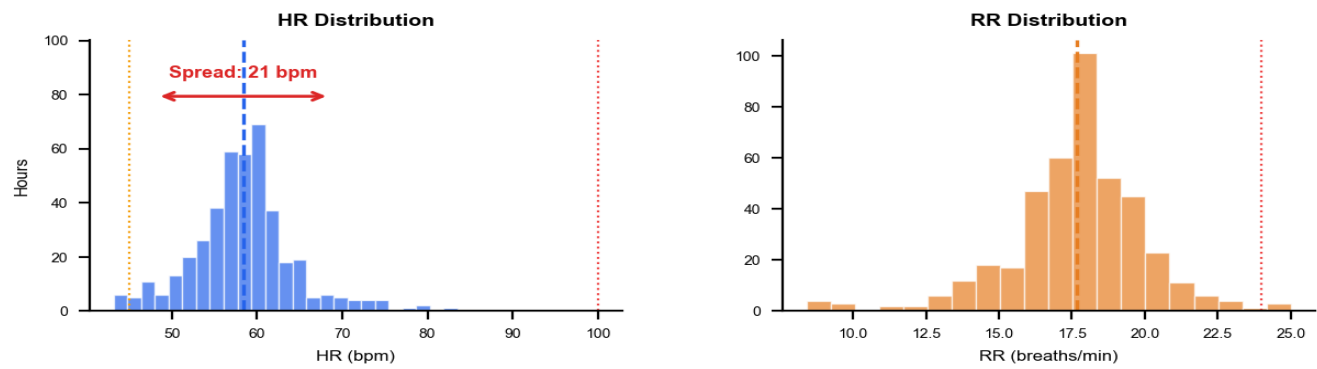
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

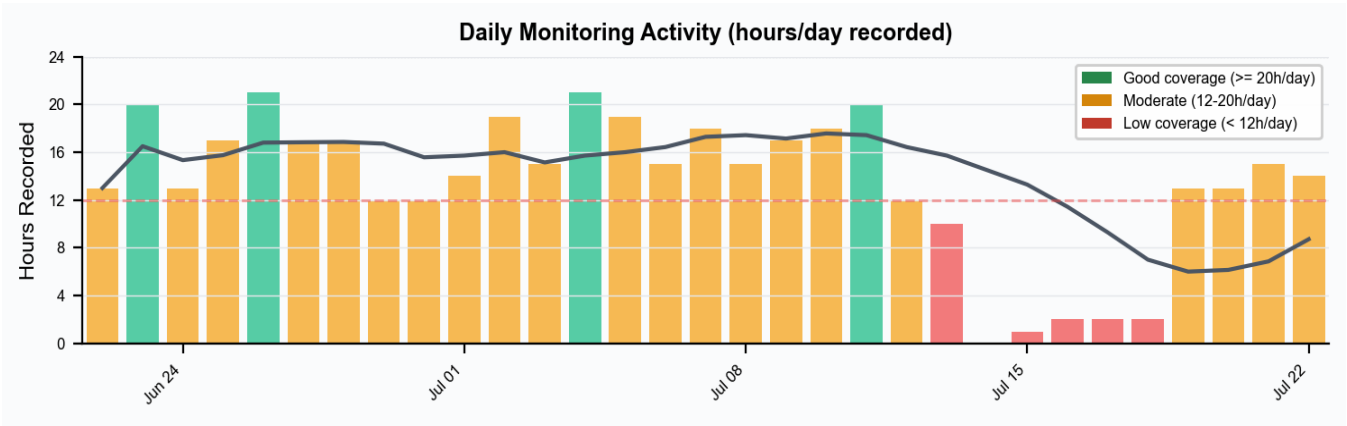
- Very Low HR < 40 bpm
Very High HR > 110 bpm
- Low HR < 45 bpm
Elevated Breathing > 24 brpm
- Elevated HR > 95 bpm
High Breathing > 30 brpm
- High HR > 100 bpm
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.